



Osteoarthritis and Cartilage

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Guide for authors

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OSTEOARTHRITIS AND CARTILAGE is the official journal of the Osteoarthritis Research Society International. The Journal serves as a focal point and forum for the exchange of ideas for the many kinds of scientists, specialists and practitioners concerned with osteoarthritis and cartilage. One of its aims is to foster the cross-fertilization between clinical and basic sciences of the various disciplines involved. *Osteoarthritis and Cartilage* publishes manuscripts representing originality and scientific excellence on all aspects of osteoarthritis, cartilage and cartilage repair. The Journal welcomes clinical, epidemiological, experimental, translational and other types of research relevant to osteoarthritis and cartilage. *Osteoarthritis and Cartilage* aims for an independent, fair and rapid peer review of submitted manuscripts.

Submission of Manuscripts

Osteoarthritis and Cartilage extends the Submission Fee to include Letters to the Editor

The Osteoarthritis and Cartilage Submission Fee of USD \$50, introduced in January 2025, will now also be charged for submitted Letters to the Editor. The Submission Fee will continue to be waived for Editorials, Invited Reviews and Year in Review papers. Note that the Submission Fee will apply to all authors of eligible article types whether they publish subscription or Open Access.

We hope this change will improve the quality of Letters to the Editor submitted and reduce the large volume of out-of-scope submitted Letters.

Manuscripts are to be submitted through the Editorial Manager on-line submission and peer-review system. Access to *Osteoarthritis and Cartilage* submission system is as follows:

<https://www.editorialmanager.com/OAC/default.aspx>

In order to access Editorial Manager, mouse click on Register in the upper left corner. Once you have established your Username and Password, you can sign on as an author and enter the information for submitting a new manuscript.

Prepare your manuscript for upload as follows: • Create a file containing just the title page and abstract

- Create a file containing the main text of the manuscript, including all references, excluding the title page and abstract
- Do not include figures within the main text file, but supply these as separate image files

Please note that Osteoarthritis and Cartilage authors are now able to enhance their submissions by employing the new Content Innovation Tools described in more detail in Section 1.7.13 of this Guide for Authors. Please follow the instructions outlined in that section to use the new Tools.

Please read and follow the instructions to authors outlined below. Failure to follow these instructions will delay processing of the manuscript.

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1. MANUSCRIPT FORMAT

The maximum word count listed for each type of article excludes title page, abstract, tables, figure legends, acknowledgements and contributions, and references.

Manuscript format should comply with the International Committee of Medical Journal Editors' 'Uniform Requirements for Manuscripts Submitted to Biomedical Journals' (<http://www.icmje.org>).

In all matters of style, please consult the Manual of Style (ed. 8) published by the American Medical Association. Number each page sequentially and add line numbers, including the title page, abstract, text, references, figure legends, and tables. Authors are responsible for providing a manuscript written in clear English. Delay or rejection may result when papers are poorly written and in need of extensive editing. Where appropriate, authors should obtain the help of an individual or organization competent in Medical Scientific English, where English is the primary language.

Additional points: insert an extra blank line at the end of headings and paragraphs; type text without end of line hyphenation, except for compound words; be consistent with punctuation and only insert a single space between words and after punctuation.

Avoid abbreviations whenever possible, and never use unfamiliar **abbreviations**. When use of an abbreviation is customary, list the full word on its first appearance followed by the abbreviation in parentheses. Once identified, the abbreviation should be used consistently throughout the text.

Measurements should be expressed in metric units wherever possible, and along with physical and chemical quantities, should be abbreviated as recommended in the instructions to authors of the current volume of *Journal of Biological Chemistry*. **Symbols of units of measurement must accord with the Système International (SI)**. Abbreviations for SI units and statistical terms are those in Baron DN (ed.) *Units, Symbols and Abbreviations: A Guide for Medical and Scientific Editors and Authors*, 5th ed. London: Royal Society of Medicine Press. Preferred alternative units may be given in parentheses.

Generic names should be used for drugs. When proprietary brands are used in research, include the brand name and the name of the manufacturer in parentheses at the first mention of the generic drug name in the methods section.

To enhance the reliability and value of published research literature, we promote transparent and accurate reporting by encouraging the wider use of robust reporting guidelines. This is consistent with what is proposed by the Equator Network. To enhance author transparency, **we advocate the use of appropriate guidelines for reporting particular studies and include the appropriate checklist in your publication** from the <https://www.equator-network.org/>.

1.1. Ten Recommendations for OAC Manuscript Preparation

Ten recommendations for Osteoarthritis and Cartilage manuscript preparation, with special attention to enhancing Statistical validity.

These recommendations are intended to provide a framework for a clear, complete, and unambiguous presentation of data, methods, and results. It is of paramount importance, and one of the tenets of good research that published studies and experiments can be reproduced, and that the description of their outcome includes sufficient information to allow a reasonable assessment of unavoidable limitations and weaknesses. In one word: transparency.

These recommendations describe what information should be presented to the reader, and how, in order to facilitate the interpretation of the findings; it is not to specify how authors should evaluate and interpret their data. Additional guidelines and details for specific types of research are available elsewhere in this Guide for Authors.

1. State the research question and the purpose of the study. Is the ambition to describe an observation, to generate hypotheses or to estimate parameters of interest?
2. Describe the source of study participants, patients, cadavers, animals, tissues, cell lines, etc., and how many of these units that have been included in the study. What inclusion criteria did you use? How representative is your sample? To what population do you wish to generalize the findings of your study? Note that direct generalization to other cases than those studied requires information on the variability between independent cases. If all observations have been sampled from one subject, animal or cell line, direct generalization cannot be made beyond this.
3. When observations can be presented individually, either numerically (ex. table with raw data) or graphically (ex. points in figures), this should be preferred. In some cases such presentation could in itself be sufficient, in other cases it may complement the summary measures in the manuscript or in a web supplement. When fewer than 20 observations are presented, graphical representations should display individual data points (e.g. dot plot).
4. When presenting descriptive data in aggregated form, always provide the number of included observations (n) as well as adequate measures of central tendency (mean, median, etc.) and dispersion (standard deviation, range, etc.). If repeated measurements or replicates are used, present both the number of independent samples and the number of repeated observations per independent sample.
5. Describe all statistical methods in the Methods section, using well recognized terms such as Student's or Satterthwaite's t-test rather than names that are unique to a particular statistical software package such as "independent groups t-test". You should always identify the statistical software and version used.
6. The validity of the results from statistical models relies on certain assumptions being fulfilled. For example, Student's t-test is based on the assumptions of independent observations, Gaussian distribution and homogeneous variance.

Describe in the statistical methods section whether you have examined if these assumptions are fulfilled, how you performed this investigation, and what the results were. When violations of the assumptions are detected, a change to an alternative method or data transformation may be necessary to ensure validity.

7. Generalizations from observed data are often made using statistical models. We recommend that results from statistical analysis are presented with a confidence interval for the quantity of interest. The quantity of interest is typically a difference between means of two groups, risk ratio, hazard ratio, slope of linear regression etc. It is not sufficient to report confidence intervals for within-group parameters, if the conclusions are made about between-group differences. Confidence intervals provide more information on the inferential uncertainty than is included in a p-value because they describe a range of plausible and interpretable values. It is important to recognize that this range of plausible values represents the uncertainty in a generalization, not the dispersion of observed data. While standard deviations and ranges can be used to describe dispersion, confidence intervals and standard error of the means represent uncertainty. When this uncertainty is presented in text and tables, or graphically with error bars, *Osteoarthritis and Cartilage* recommends using 95% confidence intervals instead of S.E.M.

8. Statistical analysis can also lead to calculation of p-values. P-values describe the inferential uncertainty in terms of risk of a false positive conclusion. It should be recognized that: (1) tested hypotheses always relate to the generalization of an observation, never to the observation itself, (2) that merely because a finding is "statistically significant" does not necessarily imply that it has practical or clinical importance, and (3) that failure to achieve statistical significance in a test does not necessarily indicate similarity. Thus, if any p-values are reported they should be accompanied with an interval estimate of the quantity of interest (ex. confidence interval for mean difference between the groups).

Do not use "statistical significance" (or "significance") to classify or describe your results, as such a label provides no additional information above what is included in the confidence interval (or a p-value if reported). Instead, interpret the values included in the confidence intervals reported and interpret their biological or clinical relevance in light of potential biases present in the study. For further guidance, please see:

<https://www.tandfonline.com/doi/full/10.1080/00031305.2019.1583913>.

9. Please avoid common misconceptions:

a) Statistical nonsignificance ($p > 0.05$, ns, or NS) is NOT evidence of equivalence, non-inferiority, or "no difference".

b) Statistical significance is NOT evidence of clinical or practical relevance. This must be shown by other means than p-values, and the actual relevance of the findings must be clearly articulated.

c) Correcting p-values for the multiple testing of X groups does NOT generally solve the multiplicity problems inherent in testing many endpoints. When using Bonferroni (or another) correction, a strategy for addressing multiplicity issues should be presented and its consequences for the conclusions should be explained.

d) Repeated or multiple measurements per individual are NOT statistically independent (not even if their intraclass correlation coefficient is 0). The statistical analysis needs to properly account for the correlation between the observations.

10. The level of statistical rigor (and remaining inferential uncertainty in the results) should be in parity with the purpose of the study and the author's conclusions. For example, a confirmatory randomized clinical trial has little room for multiplicity issues arising from the testing of multiple endpoints. If such issues exist, they should be properly addressed already in the design of the trial. Hypothesis generating studies, on the other hand, can be analyzed without concerns for multiplicity, and case-reports (studies with very small n) may be entirely descriptive with no need for evaluation of inferential uncertainty.

A brief description of the analysis strategy in the statistical methods section will facilitate the reading of the manuscript. Authors are strongly recommended to save statistical code, output from data analyses, and raw data and raw images

for possible review, should the editors request this.

Adherence to these recommendations will greatly facilitate the review of manuscripts, decrease the likelihood of multiple revisions, and improve the chances of acceptance for publication.

Recommended reading:

1. Ronald L. Wasserstein and Allen L. Schirm and Nicole A. Lazar, Moving to a World Beyond " $p < 0.05$ ". The American Statistician 2019; 73(sup1):1-19
2. Cumming G, Fidler F, Vaux DL. Error bars in experimental biology. J Cell Biol 2007;177:7-11.
3. Vaux D. Ten rules for the presentation and interpretation of data in scientific publications. Australian Biochemist 2008;39:37-9.
4. Ranstam J. Sampling uncertainty in medical research. Osteoarthritis Cartilage 2009;17:1416-9.
5. Ranstam J. Reporting laboratory experiments. Osteoarthritis Cartilage 2010;18:3-4.
6. Ranstam J, Lohmander LS. What's in a number or in a picture? Osteoarthritis Cartilage 2010;18:1003-5.
7. Ranstam J, Lohmander LS. Ten recommendations for OAC manuscript preparation, common for all types of studies. Osteoarthritis Cartilage 2011;19:1079-80.
8. Turkiewicz A, Luta G, Hughes HV, Ranstam J. Statistical mistakes and how to avoid them - lessons learned from the reproducibility crisis.

1.1.1 Manuscripts reporting Clinical Trials

All randomized controlled trials submitted for publication in the journal should follow the Consolidated Standards of Reporting Trials (CONSORT) guidelines, and include a completed CONSORT flow chart as a manuscript figure. Please refer to the CONSORT statement website at <http://www.consort-statement.org> for more information. A copy of the study protocol and statistical analysis plan should be included with the submitted manuscript, together with a completed CONSORT checklist. This checklist can be found for downloading on the CONSORT website.

Osteoarthritis and Cartilage has adopted the proposal from the International Committee of Medical Journal Editors (ICMJE) which requires, as a condition of consideration for publication of clinical trials, registration in a public trials registry such as <http://www.clinicaltrials.gov/> or <http://www.isrctn.com/>. All trials that began enrolling patients after July 1, 2005, must have been registered at or before onset of patient enrollment. Any trial which was still seeing patients on September 13, 2005, should have been registered before September 13, 2005. If the trial was complete before September 13, 2005, the trial should be registered before manuscript submission. The clinical trial registration number should be included at the end of the abstract of the article. Clinical trial manuscripts that do not fulfill these criteria will not enter the editorial process, but will be returned to the authors.

For this purpose, a *clinical trial* is defined as any research project that prospectively assigns human participants or groups of humans to one or more health-related interventions to evaluate the effects of health outcomes. Health-related interventions include any intervention used to modify a biomedical or health-related outcome (for example drugs, surgical procedures, devices, behavioral treatments, dietary interventions, and process-of-care changes). Health outcomes include any biomedical or health-related measures obtained in patients or participants, including pharmacokinetic measures and adverse events. Purely observational studies (those in which the assignment of the medical intervention is not at the discretion of the investigator) do not currently require registration. Further information can be found at <http://www.icmje.org>. *Disclosure of Clinical Trial Results*. In line with the position of the International Committee of Medical Journal Editors, the journal will not consider results posted in the same clinical trials registry in

which primary registration resides to be prior publication if the results posted are presented in the form of a brief structured (500 words) abstract or table. However, divulging results in other circumstances (e.g., investors' meetings) is discouraged and may jeopardize consideration of the manuscript. Authors should fully disclose all posting in registries of results of the same or closely related work.

1.1.2 Reporting observational studies in epidemiology

Manuscripts submitted for publication in the journal reporting observational studies in epidemiology should conform with the recommendations of the STROBE initiative (STrengthening the Reporting of OBservational studies in Epidemiology). Checklists for different types of observational studies are available at <https://www.strobe-statement.org/index.php?id=available-checklists>. Manuscripts reporting observational studies based on electronic register data should comply with RECORD guidelines (<https://www.record-statement.org/>). A completed checklist should be submitted with the manuscript. Adherence to these recommendations will greatly facilitate the review of manuscripts, decrease the likelihood of multiple revisions, and improve the chances of acceptance for publication.

1.1.3 Reporting animal and laboratory experiments

To fully understand the context, methods, data and conclusions that relate to an experiment, the reader must have access to appropriate background information. The experiment should be described in a way that makes it possible for the reader to repeat it. A clear description of the chosen study design is necessary for the reader's understanding of both the experiment and the statistical analysis of the data generated by the experiment.

Analysis units. Describe the experimental unit clearly. This is usually the smallest unit that can be independently randomized to a group, i.e. it should be possible to randomize any two experimental units to different groups. The experimental unit should also be the statistical analysis unit. Additional information relevant to the analysis units in basic science can be found in <https://elifesciences.org/articles/32486>.

Experimental design. Describe the randomization procedure (or another procedure used to allocate experimental units to groups), and present the number of randomized units, independent units and the total number of units in the study. Describe clearly which units are dependent and which are independent and how potential dependence was handled in the statistical analysis. Describe if the conduct of experiment and outcome assessment were done blinded or not. Formal experimental designs, like randomized block, latin square, split-plot, etc., have been developed and are described in a number of statistical textbooks. State clearly if one of these formal designs are used. If this is not the case, describe and explain the used design in detail. A useful list of important issues in design and analysis of laboratory studies can be found in RIPOSTE (<https://elifesciences.org/articles/05519>).

Additional information relevant to the high quality reporting of animal model studies in osteoarthritis research may be found in the following OAC publications: [Reference 1](#)), [Reference 2](#) and [Reference 3](#)

Osteoarthritis and Cartilage supports the ARRIVE (Animal Research: Reporting *In Vivo* Experiments) guidelines to improve standards of reporting of animal experiments and ensure that the data can be fully evaluated and utilized [<https://journals.plos.org/plosbiology/article?id=10.1371/journal.pbio.1000412>] (Kilkenny C, Browne WJ, Cuthill IC, Emerson M, Altman DG (2010) Improving Bioscience Research Reporting: The ARRIVE Guidelines for Reporting Animal Research. *PLoS Biol* 8(6): e1000412. doi:10.1371/journal.pbio.1000412). A completed ARRIVE checklist should be submitted with the manuscript- the checklist form can be found at <https://arriveguidelines.org/sites/arrive/files/documents/Author%20Checklist%20-%20E10%20only.pdf>. Adherence to these recommendations will greatly facilitate the review of manuscripts, decrease the likelihood of multiple revisions, and improve the chances of acceptance for publication.

The design and analysis of experiments using microarray technology poses some specific challenges, and authors intending to submit such studies to *Osteoarthritis and Cartilage* are recommended to read the article "Churchill GA. Fundamentals of experimental design for cDNA microarrays. *Nature Genetics suppl.* 2002;32:490-5. Doi10.1038/ng1031". Many journals, including *Osteoarthritis and Cartilage*, now also require, as an example, that authors reporting microarray-based experiments comply with the Minimum Information about a Microarray Experiment (MIAME) checklist

as a prerequisite for publication. Similar minimum information guidelines are available for reporting proteomics (MIAPE <http://www.psdev.info/miape/>) and other types of 'omics' studies. For further reading on evolving reporting guidelines, see Minimum reporting guidelines for biological and biomedical investigations (MIBBI) (<http://mibbi.sourceforge.net/projects/MIAME.shtml>).

1.1.4 Reporting studies that utilize machine learning or artificial intelligence

For submissions employing machine learning (ML) or artificial intelligence (AI) methods, we recognize that the community has not reached consensus on appropriate reporting requirements. We emphasize, however, that as a biomedical journal it is our mission to publish research that furthers the understanding of the biology and clinical care of osteoarthritis. As such, it is imperative that any prediction model be sufficiently open and accessible to permit external validation, and we encourage authors to provide applicable data sets or executable code for review.

For authors' convenience, we endorse the CLAIM checklist, <https://pubs.rsna.org/doi/full/10.1148/ryai.2020200029>. However, until such a consensus has been reached, we will accept adherence to any of the published guidelines proposed by mainstream organizations, such as the RSNA (Bluemke DA et al., <https://doi.org/10.1148/radiol.2019192515>), the NeurIPS Reproducibility Program (<https://arxiv.org/pdf/2003.12206.pdf>), or the TRIPOD Statement (<https://bmcmmedicine.biomedcentral.com/articles/10.1186/s12916-014-0241-z>).

1.1.5 Reporting Mendelian Randomisation studies

Mendelian Randomisation (MR) analyses are commonly categorized into two forms in the literature: one-sample (or single-sample or individual-level) and two-sample (or summary-level). Initial MR analyses, mainly published before 2014, used individual-level data, where genotype, exposure, and outcome data were all available in the same population. Modern summary-level techniques simply require summary statistics from a GWAS of the exposure and outcome, not on the same sample but assuming the GWAS populations are generated from the same underlying population. A summary-level MR analysis must ensure that the two study populations are drawn from the same underlying population, or that the instrument-exposure association is the same in both samples, while reducing sample overlap. Due to chance connections between the instrument and confounders of the exposure-outcome relationship, overlap in samples can correlate SNP-exposure and SNP-outcome estimates. (see Hartley et al. <https://doi.org/10.1002/jbm4.10675>)

We are only interested in two sample MR studies that investigate something novel related to OA

- if there are one or more MRs already published on the topic, the authors need to explain what is novel or relevant in their study.
- if there are no clinical or mechanistic insights (e.g. specific pathways involved in the relationship between OA and a given trait) derived from the study, the manuscript might get a lower priority, therefore, we encourage authors to investigate or address such aspects.
- we are not interested in MR studies that investigate the bidirectional causal links between OA and OA symptoms unless there are clear mechanistic or clinical insights.
- in the cover letter, please provide a strong rationale for investigating the causal relationship between two traits using MR. if there are no clinical or mechanistic insights (e.g. specific pathways involved in the relationship between OA and a given trait) derived from the study, the manuscript might get a lower priority, therefore, we encourage authors to investigate or address such aspects, including providing prior evidence supporting that the exposure may be associated with OA.
- selection of instrumental variables should be based upon appropriate biologic basis or a prior well-powered GWAS, with appropriate supporting evidence for the selection.
- in the cover letter please specify how you have ensured that the instrumental variables (IVs) used in your study are associated with OA solely through the studied exposure (and not through any other factors) and specifically how you

have ensured that all IVs are independent of confounders.

For example, if your exposure is known to be associated with BMI, and you are investigating knee OA, then please highlight how you have ensured that all IVs are independent of BMI. If you have not ensured independence from BMI because you have included it as part of a multivariable Mendelian Randomization showing that any causative links between knee OA and your chosen exposure are not mediated by BMI, then please indicate that clearly in your cover letter.

- you also need to succinctly state in your cover letter what is the translational value or the key mechanistic insights of your MR study

In addition, we require authors to fill out the [STROBE-MR checklist](#), and explain that studies should be sufficiently powered and be innovative as per the points described above.

1.2 Full length original research article

Full length original articles should amount to no more than 4000 words, 8 figures and tables, and 50 references. Each of the following sections should be included in the manuscript in this order: **Title page, Abstract, Introduction, Method, Results, Discussion, Acknowledgments, Author contributions, Role of the funding source, Conflict of interest, References, Figure legends, each individual Table, each individual Figure.** Specific recommendations relating to these manuscript sections follow below.

1.3 Brief report of original research

Brief reports may be submitted for the rapid communication of results of significant interest and novelty. Brief reports should follow the general manuscript format described above, but not exceed an abstract of 250 words, a main text of 2000 words, 15 references, and 2 figures and/or tables.

1.4 Editorial and Commentary

The purpose of an editorial is to stimulate discussion and thought in a brief format. As such, it may contain personal opinion and comment as supported by evidence. Editorials often provide perspectives linked to an article in the same issue. Editorials also provide a forum for the editors to inform readers of updates or changes in editorial policies. Submissions of Editorials are typically invited by the Editors.

The purpose of a Commentary is similar to an Editorial, however these may be unsolicited and need not be invited contributions. They typically involve areas of interest that may be controversial or for which the evidence base is less secure. Authors are encouraged to obtain approval from the Editors prior to submitting Commentaries. Papers accepted as Commentaries may be sent to content authorities in the field to provide short (250 words) responses. These mini-commentaries will be published in conjunction with the original Commentary, and will serve both as "open" peer review and to stimulate discussion in the community.

Editorials and Commentaries should be no longer than 1500 words with a maximum of 2 figures and/or tables and 15 references. Recommendations relating to relevant sections of this article type follow below.

1.5 Narrative Reviews, Systematic reviews and Meta-Analyses'

The suggested word count is 4000, a maximum of 8 tables and/or figures and 100 references. Reviews should include a structured abstract. Recommendations relating to relevant sections of this manuscript type follow below.

Reviews may come in different formats. The Systematic Review format is preferred wherever possible and appropriate. Systematic reviews and meta-analyses should be reported following the recommendations of the PRISMA statement (<http://www.prisma-statement.org>). *Osteoarthritis and Cartilage* strongly recommends that a completed PRISMA checklist accompanies the submitted manuscript. Study protocols must be prospectively registered before submission. A study protocol for a systematic review and meta-analysis and network MA investigating the effect on humans and most scoping reviews be registered in the public database PROSPERO <http://www.crd.york.ac.uk/prospero/>. SYRCLE could be

used for ANIMAL studies <https://norecopa.no/3r-guide/systematic-review-centre-for-laboratory-animal-experimentation-syracle/>. For systematic reviews on other topics, including meta-epidemiology/methodology, we strongly recommend pre-registration in Open Science Framework <https://osf.io/>. Following these recommendations will greatly facilitate the review of manuscripts, decrease the likelihood of multiple revisions, and improve the chances of acceptance for publication.

Narrative reviews are published at the discretion of the editors-in-chief and are usually invited. If you feel that a narrative review is an appropriate format for your uninvited article, please first contact the editorial office – oac@elsevier.com – who will discuss your idea with the editors-in-chief. Note that their agreement for you to submit a narrative review does not imply acceptance and the article will still go through the regular peer review process.

1.6 Letter to the Editor

Comments regarding articles published in the Journal, or other current matters, are solicited and should be submitted as 'Letter to the Editor'. Such Letters, which should not be original research communications (see *Brief Report* the appropriate format for such manuscripts), are subject to editorial review. When a published article is subjected to comment or criticism, the authors of that article will be invited to submit a letter of reply.

Letters are no more than 800 words, no abstract, no tables or figures, and maximum 8 references. Recommendations relating to relevant sections of this form of article follow below.

1.7 Supplementary data

Osteoarthritis and Cartilage now accepts electronic supplementary material to support and enhance your scientific research. Supplementary files offer the author additional possibilities to publish supporting applications, movies, animation sequences, high-resolution images, background methods and datasets, sound clips and more. Supplementary files supplied will be published alongside the electronic version of your article, including ScienceDirect:

<http://www.sciencedirect.com>. In order to ensure that your submitted material is directly usable, please ensure that data is provided in one of our recommended file formats. Authors should submit the material in electronic format together with the article and supply a concise and descriptive caption for each file. For more detailed instructions please visit: <https://www.elsevier.com/artwork>.

1.8 RESEARCH DATA

This journal encourages and enables you to share data that supports your research publication where appropriate, and enables you to interlink the data with your published articles. Research data refers to the results of observations or experimentation that validate research findings. To facilitate reproducibility and data reuse, this journal also encourages you to share your software, code, models, algorithms, protocols, methods and other useful materials related to the project.

Below are a number of ways in which you can associate data with your article or make a statement about the availability of your data when submitting your manuscript. If you are sharing data in one of these ways, you are encouraged to cite the data in your manuscript and reference list. Please refer to the "References" section for more information about data citation. For more information on depositing, sharing and using research data and other relevant research materials, visit the [research data](#) page.

1.8.1 Data linking

If you have made your research data available in a data repository, you can link your article directly to the dataset. Elsevier collaborates with a number of repositories to link articles on ScienceDirect with relevant repositories, giving readers access to underlying data that gives them a better understanding of the research described.

There are different ways to link your datasets to your article.

When available, you can directly link your dataset to your article by providing the relevant information in the submission system. For more information, visit the [database linking page](#).

For [supported data repositories](#) a repository banner will automatically appear next to your published article on ScienceDirect.

In addition, you can link to relevant data or entities through identifiers within the text of your manuscript, using the following format: Database: xxxx (e.g., TAIR: AT1G01020; CCDC: 734053; PDB: 1XFN).

1.8.2 Mendeley Data

This journal supports Mendeley Data, enabling you to deposit any research data (including raw and processed data, video, code, software, algorithms, protocols, and methods) associated with your manuscript in a free-to-use, open access repository. Before submitting your article, you can deposit the relevant datasets to *Mendeley Data*. Please include the DOI of the deposited dataset(s) in your main manuscript file. The datasets will be listed and directly accessible to readers next to your published article online.

For more information, visit the [Mendeley Data for journals page](#).

1.8.3 Data statement

To foster transparency, we encourage you to state the availability of your data in your submission. This may be a requirement of your funding body or institution.

If your data is unavailable to access or unsuitable to post, you will have the opportunity to indicate why during the submission process, for example by stating that the research data is confidential. The statement will appear with your published article on ScienceDirect. For more information, visit the [Data statement](#) page.

1.9 Manuscript sections

1.9.1 Title page

Title page should include affiliations, ORCID ID and email addresses for each co-author, and full contact details for the corresponding author.

When appropriate, include a separate running title.

1.9.2 Abstracts

Abstracts should be no more than 350 words. The abstract should be structured into sections, at least including the following: (1) Objective; (2) Design - if clinical to include setting, selection of patients, details on the intervention, outcome measures, etc.; if laboratory research to include details on materials and methods; (3) Results; (4) Conclusions. For further detail on how to construct an abstract please refer to the guidelines published in the *Journal of the American Medical Association* (JAMA 2004;291:125-9). Note that supplying a structured abstract is required for all relevant article types, including narrative reviews.

Keywords

Immediately after the abstract, provide a list of 3-6 keywords, using English spelling and avoiding general and plural terms and multiple concepts (avoid, for example, `and?', `of?'). Be sparing with abbreviations: only abbreviations firmly established in the field may be eligible. These keywords will be used for indexing purposes.

Running headline

A running title of not more than 40 characters (including spaces), suitable for page headings, should be provided at the bottom of the title page if the full title is longer than 40 characters.

1.9.3 Introduction

Introduction should be brief, to the point, and contain the background that motivated the study. You may assume that the reader is familiar with e.g. the fact that osteoarthritis is a common joint disease, there's no need to repeat that in this journal.

1.9.4 Method

Methods shall contain details relevant to the conduct and interpretation of the study. This includes a description of statistical methods sufficiently detailed so that an investigator with access to the data can verify the results (see section below on statistics). Classification criteria should be reported on patients (where relevant) by a brief description of the clinical features of patients, and by reference to the criteria used. Use of subheadings that aid clarity is encouraged.

1.9.5 Results

Avoid undue repetition of data in text and tables. Brief comments on the significance of the results is appropriate, but broader aspects of interpretation is reserved for discussion. Use of subheadings to aid clarity is encouraged.

It should be recognized in the results presentation that a statistically significant effect or difference not necessarily is of interest, it may be too small to be relevant. It is therefore better to specify the effect size/standardized response mean, and presenting the uncertainty with a 95% confidence interval, than describing an effect as statistically significant, or not statistically significant. P-values should be presented numerically, without categorization, e.g. write $p = 0.15$, not ns, and $p = 0.03$, not $p < 0.05$. When computer printout says $p = 0.0000$, write $p < 0.0001$. Confidence intervals should be presented as (lower limit, upper limit).

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Discussion section should contain a concise discussion of the findings in context of relevant published data. Which of your results are confirmatory, which are novel? Specifically, how do your results advance this field of research? A section on limitations of interpretation of results due to the selection of methods, materials or patients is often recommended. Avoid lengthy extrapolation and speculation.

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Reference to a book:

[3] W. Strunk Jr., E.B. White, The Elements of Style, fourth ed., Longman, New York, 2000.

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[4] G.R. Mettam, L.B. Adams, How to prepare an electronic version of your article, in: B.S. Jones, R.Z. Smith (Eds.), Introduction to the Electronic Age, E-Publishing Inc., New York, 2020, pp. 281 – 304.

Reference to a website:

[5] Cancer Research UK, Cancer statistics reports for the UK.
<http://www.cancerresearchuk.org/aboutcancer/statistics/cancerstatsreport/>, 2023; (Accessed 13 March, 2023).

Reference to a dataset:

[6] M. Oguro, S. Imahiro, S. Saito, T. Nakashizuka, Mortality data for Japanese oak wilt disease and surrounding forest compositions [dataset], Mendeley Data, v1, 2015. <https://doi.org/10.1234/abc12nb39r.1>.

Reference to software:

[7] E. Coon, M. Berndt, A. Jan, D. Svyatsky, A. Atchley, E. Kikinzon, D. Harp, G. Manzini, E. Shelef, K. Lipnikov, R. Garimella, C. Xu, D. Moulton, S. Karra, S. Painter, E. Jafarov, S. Molins, Advanced Terrestrial Simulator (ATS) v0.88 (Version 0.88) [software], Zenodo, March 25, 2020. <https://doi.org/10.1234/zenodo.3727209>.

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